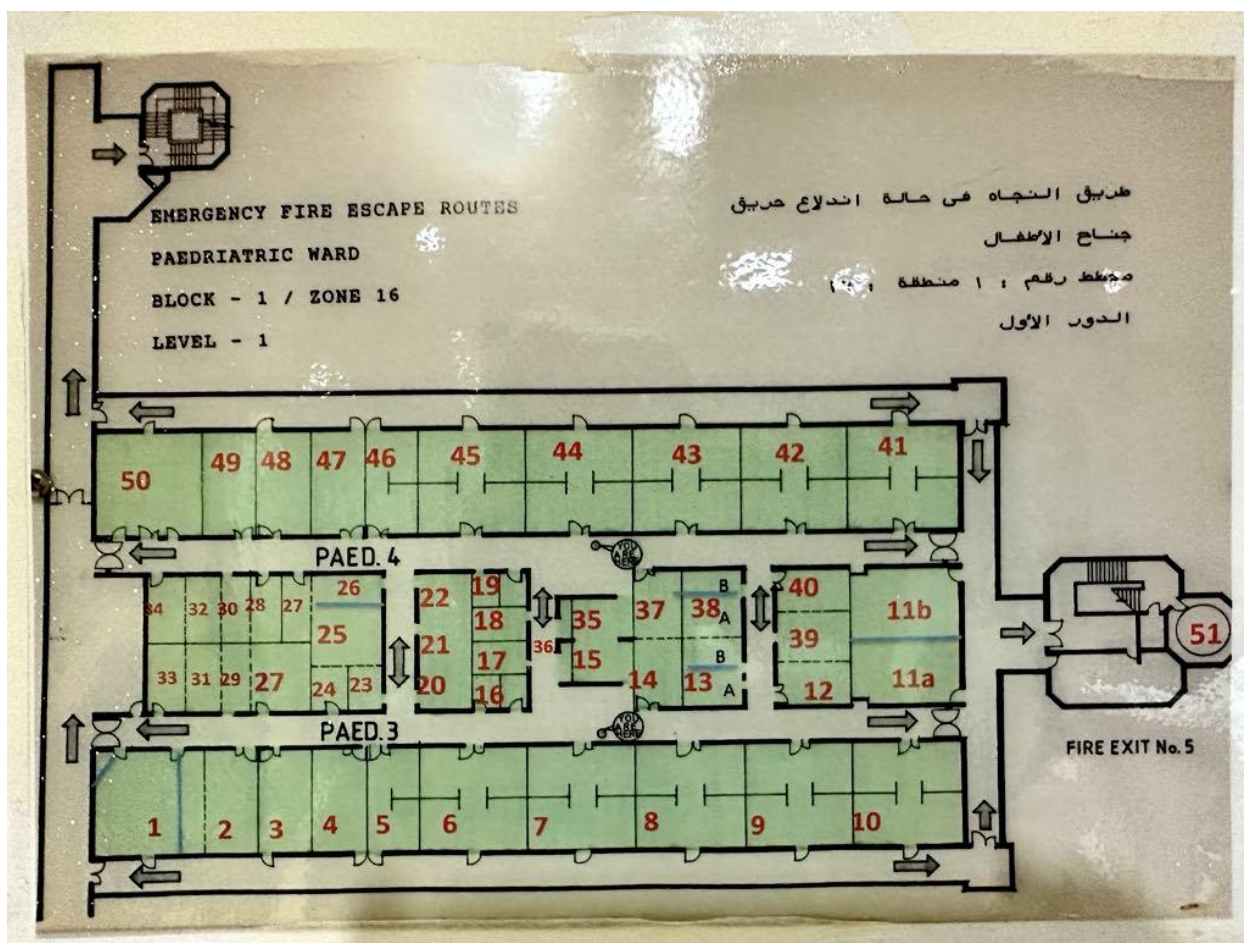






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Sultanate of Oman
Ministry of Health
The Royal Hospital
Department of Fire Safety

Code: FS-SOP-2-Vers.3.0

Effective Date: 04/07/2021

Target Review Date: 31/12/2024

Title: General Fire Evacuation Procedure

إجراءات خطة الإخلاء

GENERAL LOCAL EVACUATION PROCEDURE

1. Introduction

This document is created by fire prevention department which includes the action to be taken by all staff in the event of fire and the arrangements for calling the fire safety officers in Royal Hospital on **5024 or 9409**. The purpose of this document is to give the staff a clear and concise instructions of the routine to be followed in case of fire. The responsibility of the fire safety officer is to ensure that staff and relevant persons in the ward can read this document and familiar with its contents.

The primary aim of a hospital facility is to not evacuate patients unless absolutely necessary. Hence, special attention should be focused on proper prevention and suppression techniques to avoid this worst-case scenario. That being said, evacuation training and preparedness is importance to avoid and/or minimize loss of life.

This document is only applicable for Royal Hospital, and it provides a guide on hospital evacuation. It details the tasks and responsibilities of each member of the evacuation team. The personnel suggested in this plan vary depending on the particular **WARD/ DEPARTMENT** for which the evacuation plan is being developed.

2. Types of evacuation

There are two types of evacuation, Total evacuation where all the people in the ward / area should be evacuated because the spread of fire cannot be brought under control and that can affect the health & safety of the people in the hospital.

The second type is the partial evacuation – which we are implementing in Royal Hospital –in this type of the fire risk can be isolated to prevent or slow down its spread until it is brought under control. Partial evacuation in Royal Hospital is

3. Assembly Point

In the event of a fire when evacuation is deemed necessary by Ward in-charge, he/she will initiate the evacuation of occupants (horizontally) to nearest initial assembly point , which is inside the Hospital building.

4. Ward Management

At all times there must be somebody in charge of the ward with responsibility for controlling and supervising activity in the event of a fire. The following staff have been nominated to perform the evacuation activities and communicate with the fire prevention department.

Responsible Staff	Duties in case of fire	Names & Contact Details
Senior Ward Nurse	Evacuate the ward as necessary with a coordination with the fire safety officer.	Name: Contact Number:
Doctor in charge	To decide the sequence of moving the patients.	Name: Contact Number:

Actions on Discovering a Fire

- 1. Raise the alarm:** Break the glass on a fire alarm call point.
- 2. Move the patient** from immediate danger.
- 3. Get out and close (but not lock) the door behind you:** this will contain the fire and smoke in one area.
- 4. Report to the nurse station** and give information about the fire to Ward in Charge. The Ward in Charge needs to phone the fire Code on (9777).

5. Move people to safety: the ward in charge needs to start evacuating the ward with coordination with the doctor in charge and fire prevention department.

- The ward in charge to decide on which access doors to use.
- Decide which patients to move first. Movement will begin with relocation of patients most affected by the emergency in the following order (subject to doctor approval):

- Patients in immediate danger**
- Ambulatory patients**
- Non-ambulatory patients**

Note: this sequence not applicable in Intensive Care Unit adult.

- The Ward in charge should have a clear idea on the method of evacuating the patients to a place of safety by using the following methods:
- Walking** – for those patients who can walk themselves out of the ward. Those patients should be supervised by the nurses/ fire officer in the ward.
 - Wheeled transport (wheelchairs or wheel beds)** - those who have restrictions to their ability to walk themselves out of the ward.

- If there are insufficient personnel to evacuate all patients request additional help and contact for additional personnel.

6. Tackle the fire if safe to do so: Send staff to fight the fire if staff are available and it is safe to do so until the Fire Officer and Civil Defence reach the location. (Remember that the fire officer on duty will take at least 3 minutes to arrive).

7. Go to the fire assembly point: Move quickly, safely, and quietly.

- Take a roll call to ensure all staff, patients and carers are present.
- If all are not present reports to Fire Officer, the number of persons missing and where last seen.

Actions on Hearing the Fire Alarm

On hearing the alarm staff must: -

1. Remain calm.
2. All staff should recognise the sound.
 - If the alarm sound is continuous, it means that there is a fire in your area.
 - If the sound is intermittent the fire is in a nearby area.
3. All staff should report to the person in charge.
4. If staff find the fire they should:
 - Gather information of the fire.
 - Follow the action as mentioned above.

Note : Each fire procedure has to contain ward drawing

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	Sultanate of Oman Ministry of Health The Royal Hospital Department of Fire Safety
Code: FS-POL-4-Vers.1.0	
Effective Date: 16/12/2021	
Target Review Date: 26/12/2024	
Title: Policy & Procedure for A Fire Evacuation Training	

<u>POLICY & PROCEDURES FOR A FIRE EVACUATION TRAINING</u> 1. POLICY The Royal Hospital shall always have in place a policy for fire evacuation training to give the staff a clear and concise instructions of the routine to be followed in case of fire. This shall be known as a policy & procedure for a fire evacuation training. 2. OBJECTIVE <u>Fire Evacuation Training (Fire drill) objectives include:</u> • Giving Royal Hospital staff an opportunity to practice emergency procedures in a simulated but safe environment. • Determining if staff understand and can carry out emergency evacuation in case of fire. • Evaluating effectiveness of evacuation procedures and determining necessary changes or adjustments to procedures to improve performance. • Complying with requirements of the local fire code and international standards. 3. FREQUENCY OF FIRE DRILL The frequency of fire drills depends in part on the requirements of the location and on fire hazards in the workplace. In departments with serious fire hazards (for example, Laboratory and Kitchen), fire drills should be conducted at least twice a year. In other departments one fire drill per year is sufficient.
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There are two types of evacuation, total evacuation and partial. Total evacuation where all the people in the hospital should be evacuated because the spread of fire cannot be brought under control and that can affect the health & safety of the people in the hospital.

The second type is the partial evacuation – which we are implementing in Royal Hospital –in this type of the fire risk can be isolated to prevent or slow down its spread until it is brought under control.

5. FIRE DRILL TRAINING PROCEDURE

- a. Discuss the scenario and prepare empty beds.
- b. Identify date and time.
- c. Observe the evacuation process and measure the time spent for evacuation.
- d. Activate the fire alarm.
- e. Search the affected area, in the event of a continuous fire alarm.
- f. Move patient who are near to the fire origin.
- g. Tackle the fire if safe to do so.
- h. Go to the assembly point (as per the evacuation plan).
- i. Ensure all people are out of the dangerous area.
- j. Prepare evacuation drill report.

6. FIRE DRILL EVALUATION

To make sure drill objectives have been met and staff have conducted themselves properly and effectively during the drill, the fire personnel will observe the drill and evaluate it immediately following the event and prepare the fire drill report accordingly.

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